

WORKING BRIEF

Deaf and DeafBlind end-of-life and palliative care

UK statutory layer, tactile communication, international comparison, lived harm and documented improvement.

Compiled 8 September 2026 from the four-prompt research batch. Sits alongside the SWL ICB Deaf/DeafBlind healthcare dossier. Unbiased working file: follow money, fulfilment and what investigators actually found. Do not wear a DEI or anti-DEI banner.

How to use this file

This is a working compilation of Prompts 1–4, not a campaign pamphlet. Cream/modelled claims from earlier SWL cost work are not restated here. Every finding is tied to a titled source, publisher, date and URL. Where evidence is thin or absent, that is stated in plain language rather than filled from general Deaf-health literature. The DeafBlind last-days record is distinct and under-researched; Prompt 2 is written to that constraint.

- **Prompt 1.** UK statutory services — NHS, hospice, DWP Special Rules, CQC and PHSO.
- **Prompt 2.** Tactile BSL, Deafblind Manual, hands-on signing, social haptics in dying care.
- **Prompt 3.** Peer-reviewed and health-system approaches outside the UK.
- **Prompt 4.** Lived harm, advocacy asks, psychological toll, and documented improvement.

Headline findings in one page

- AIS (DAPB1605, 30 June 2025) already covers dying Deaf and DeafBlind patients, including registered BSL and deafblind interpreters, promptly and without unreasonable delay. There is no separate terminally-ill protocol. Enforcement is complaint-led. CQC does not itself assure AIS.
- Two PHSO investigations name Deaf patients at the end of life: Mrs W (2015) and Alan Graham / UHB (published 11 February 2026). The 2026 case is the cleanest: three professional interpreters in 11 weeks; a 16-year-old and a 12-year-old used for prognosis and a DNACPR-type conversation.
- One CQC penalty (UHB, October 2022, £8,000) names a Deaf BSL user, finds consent taken without an interpreter, and records that the patient later died. Same Trust as Alan Graham.
- No published CQC hospice inspection names a Deaf or DeafBlind patient and attributes a poor death to missing BSL or tactile access. No published DWP or PHSO finding names a Deaf Special Rules claimant — structurally, accepted SREL claims skip assessment.
- No published UK hospice or NHS palliative protocol exists for tactile BSL, Deafblind Manual, hands-on signing or social haptics with a dying DeafBlind patient. Hospital haptics exist in Nordic SHC literature (Lahtinen/Palmer); they are not an NHS or Hospice UK standard.
- RIDB supply is the binding constraint: government baseline 8 (June 2026 grant); live NRCPD count 19 on 5 September 2026 (London 1; SW / East Midlands / Yorkshire & Humber 0); grant target 68. Communicator-guides are social-care roles, not an on-call hospice bench.
- No large international quantitative series measures hospice outcomes for Deaf or DeafBlind patients. Transferable practice is small and local: Minnesota Deaf hospice volunteers (1998–), Ottawa Deaf Palliative Care Team (2009–), Jacksonville ASL CARES (includes deaf-blind volunteers), France

UASS/UNISS (23 LSF-fluent hospital units), Germany Deaf Pal (Landshut palliative ward), Canada and Australia sign-language palliative video packs.

- Imogen Nunn's Prevention of Future Deaths reports (March and October 2025) treat missing BSL in mental-health crisis as a threat-to-life systems problem. That is not a hospice finding. It is the only recent English coroner text that puts interpreter shortage on a statutory risk register.

1. UK statutory services — NHS, hospice and DWP

Legal baseline

The Accessible Information Standard (AIS, DAPB1605) is issued under s.250 of the Health and Social Care Act 2012. Current requirements text published and last updated 30 June 2025. It applies to every provider and commissioner of NHS care and publicly funded adult social care — trusts, ICBs, GPs, community services, and independent providers delivering NHS-funded care, which includes most English hospices when they take NHS-funded patients. Organisations must identify, record, flag, share and meet communication needs; provide support promptly; and use communication professionals (BSL and deafblind interpreters) who hold recognised registration (NRCPD or RBSLI), DBS clearance and a professional code. Longer appointments should be offered where needed.

CQC does not assure AIS; commissioners do. CQC may act where it learns needs are not being met, because AIS maps onto fundamental standards including person-centred care, dignity, consent, safety and duty of candour. CQC provider guidance last updated 10 March 2026 restates that AIS covers BSL interpretation and applies to carers and family members who themselves have communication needs. The Equality Act 2010 anticipatory reasonable-adjustment duty sits underneath. PHSO treated both AIS and the Equality Act as the standard in Alan Graham.

AIS contains no separate “terminally ill” protocol. The same five-step duty applies on a ward, in A&E at 02:00, in a hospice inpatient unit, and on a district-nursing visit. The gap is operational: dying is time-critical; the booking market is not.

NHS England, Accessible Information Standard – requirements (DAPB1605), 30 June 2025.

<https://www.england.nhs.uk/long-read/accessible-information-standard-requirements-dapb1605/>

CQC, Meeting the Accessible Information Standard, page updated 10 March 2026.

<https://www.cqc.org.uk/guidance-providers/meeting-accessible-information-standard>

1.1 NHS Trusts — AIS and terminally ill Deaf patients

PHSO — Alan Graham / University Hospitals Birmingham (11 February 2026)

Alan Graham, 75, born Deaf, BSL first language. Two admissions to Queen Elizabeth Hospital, Birmingham, June–September 2021, heart failure, died two weeks after the second admission. Across 11 weeks, professional BSL interpreters were provided on three occasions. Staff repeatedly used his grandchildren — Connor Petty, 16, and Mia, 12 — to relay treatment, prognosis and a DNACPR-type decision. On the night before he died, clinicians asked Connor to tell his Deaf mother, Jennifer Graham-Petty, that Alan might not survive the night and that CPR should not be given. PHSO found failure to comply with national guidance, AIS and the Equality Act. Missing interpreters did not change medical treatment; they caused unnecessary distress in the weeks before death and limited the daughter's ability to communicate with clinicians. Remedies: action plan, apology, £900 to each grandchild, £750 to Jennifer. PHSO raised the pattern with NHS England and RNID.

“By not consistently providing BSL interpreters to Alan, the Trust caused unnecessary distress in the weeks before his death.”

— Rebecca Hilsenrath KC, PHSO Chief Executive, 11 February 2026

PHSO news release, 11 February 2026. <https://www.ombudsman.org.uk/news-and-blog/news/teenager-had-tell-his-deaf-mother-her-father-might-die-after-hospital-failed>

PHSO — Mrs W, Dying without Dignity (20 May 2015)

Mrs W, 89, profoundly deaf, incurable bladder cancer with lung metastases. Hospital told the GP she had a short time to live. The practice wrote rather than called, then did not follow up when she did not respond. Almost two weeks later the practice manager told her son she was dying. District nursing referral a month later; she died a month after that. PHSO found the delay meant no timely palliative plan, and that the practice had “marginalised her as a profoundly deaf person.” Communication-of-prognosis failure, not a missing bedside interpreter.

PHSO, Dying without Dignity — Poor communication, 20 May 2015.

<https://www.ombudsman.org.uk/publications/dying-without-dignity/poor-communication>

Locked Out — anonymised MND case (27 November 2025)

A Deaf BSL-using wife describes her husband’s last months with motor neurone disease: repeated interpreter requests refused; urgent appointments attended without one; consent, anaesthetic and allergy checks on paper he could not read; unqualified and trainee interpreters at diagnosis; a trainee refused to interpret consent so tests stopped; the consultation letter full of errors; a competent interpreter arrived too late; he died shortly after. After death the register office refused an interpreter and told her to bring a friend; GP and coroner appointments cancelled amid payment disputes. Anonymised first-person testimony, not a named inquest. The same report states there is no specialist BSL provision for palliative and end-of-life care, and no NHS and social care strategy which allows deaf people to die with dignity and compassion with face-to-face interpreters if they are too unwell for a VRS screen. “Hospice care is not a peaceful option for deaf and deafblind people if they cannot communicate with anyone.” Recommendation 8: in-person interpretation is essential when patients are bedridden, in pain, or receiving end-of-life care.

BSL Advisory Board / Dr Joanna Atkinson, Locked Out, 27 November 2025.

<https://www.gov.uk/government/publications/bsl-user-experience-of-health-and-social-care-in-uk/locked-out-exclusion-of-deaf-and-deafblind-bsl-users-from-health-and-social-care-in-the-uk-full-report-bsl-and-english-versions>

Clinician observation and first-person account

Dr Natasha Wilcock, a Deaf doctor working in palliative care, in RNID/SignHealth Still Ignored (2025) and recycled in Locked Out: she has met patients referred to palliative care who have not understood that they will no longer receive cancer treatment — they have not understood that they are dying. BBC World Service, 28 December 2021: Francesca Bussey — her profoundly Deaf father had one month in hospital and two hours of BSL interpreting; staff put a crossed-ear sign behind the bed and shouted; she interpreted the conversation in which doctors told him he was dying.

RNID / SignHealth, Still Ignored: the fight for accessible healthcare, 2025.

<https://signhealth.org.uk/resources/still-ignored-the-fight-for-accessible-healthcare/>

BBC News, ‘The doctors came and I had to tell my father he was dying’, 28 December 2021.

<https://www.bbc.co.uk/news/stories-59733533>

ABSENT. No published national AIS compliance rate for patients on an end-of-life pathway. No Trust-level BSL fulfilment series for palliative wards. No CQC never-event for rapid-decline interpreting.

1.2 Hospices — BSL booking in emergency and rapid decline

Help the Hospices / SignHealth / Info to Go / Remark!, BMJ Supportive & Palliative Care P166 (2013): more than 70,000 people dying each year in England and Wales estimated deaf or hard of hearing; “little deaf awareness within the world of palliative care”; family used as interpreters; limited understanding of EOL choices; little hospice information in BSL. That paper is still the most-cited sector reference. Marie Curie professional guidance lists BSL as a method and tells staff to ask the employer how to access interpreters. It is generic sensory-loss advice, not a 24/7 booking SLA. Marie Curie has a BSL grief film (April 2023) and a support line via Relay UK; it does not publish BSL fulfilment data for inpatient or community palliative teams.

Documented local good practice is not a national standard. Royal Trinity Hospice (project from late 2023; Hospice UK Improving Inclusivity Award December 2025; write-up May 2026) formed a Hearing Loss Working Group after a Deaf visitor asked how the hospice supported Deaf patients and staff could not answer. Finding: no agreed BSL signs for “hospice” or “palliative care.” Actions: specialist clinical teaching; dedicated remote-interpreting devices on the inpatient unit; community staff enabled to use VRI; BSL-interpreted Talk & Tour events. St Helena Hospice, north-east Essex, publishes a BSL Live / SignVideo button and 24/7 VRI for staff — cited in Lords, 20 March 2026, as proof that access is possible. Contact-channel access is not a guaranteed face-to-face interpreter at a rapid-decline bedside.

RAD/ASLI pandemic guidance still circulating: remote interpreting is “unlikely to be suitable for end of life care communication”; decisions on continuation or withdrawal of treatment, pain management and legal steps should only be discussed with an interpreter present. Locked Out says the same. Standard NHS/hospice contracts are office-hours plus on-demand VRI fallback (Sign Solutions InterpretersLive 08:00–20:00 in several London systems; SignLive iPads in some ED/maternity stores). Rapid decline happens out of hours.

Hodson, Ede, Dean, Edwards, BMJ Supportive & Palliative Care P166, 2013.

https://spcare.bmj.com/content/3/suppl_1/a69.1

Royal Trinity Hospice, Access & Inclusion for Deaf People, 4 May 2026.

<https://www.royaltrinityhospice.london/news/improving-access-and-inclusion-for-deaf-people-at-royal-trinity-hospice>

Hospice UK innovation example — Trinity Deaf insight. <https://www.hospiceuk.org/innovation-hub/support-for-your-role/examples-of-innovation/deaf-insight>

St Helena Hospice, Contact us via a BSL interpreter. <https://sthelena.org.uk/contact-us-via-a-british-sign-language-interpreter>

Marie Curie, Communication needs in palliative care — sensory loss.

<https://www.mariecurie.org.uk/professionals/palliative-care-knowledge-zone/communication-in-care>

Deaflink, End of Life Report, Nov 2016 / PDF 2018.

<https://www.deaflink.org.uk/sites/default/files/uploads/documents/2018/02/End%20of%20Life%20Report%20-%20Deaflink%20only.pdf>

ABSENT. No published hospice-sector BSL fulfilment rate for unplanned/rapid-decline episodes. No national one-hour emergency F2F standard. Independent hospices that take no NHS money sit outside AIS unless a contract pulls them in.

1.3 DWP — Special Rules for End of Life

From 3 April 2023 the SREL test is death from a progressive disease reasonably expected within 12 months (was 6). An SR1 completed by a GP, consultant, hospice doctor or specialist nurse is the evidence. DWP clinician guidance: SREL exists so people get faster, higher awards and avoid a medical assessment. Official PIP page: “You will not need to go to a face-to-face assessment.” Award rate for

SREL PIP claims is 99%. Enhanced daily living is automatic; mobility is considered on evidence. A third party can claim; the patient does not have to be told the prognosis.

Consequence for BSL: if the SR1 is accepted, the Deaf claimant never attends a PIP or WCA, so “interpreter at the assessment” is usually not the live issue. The live issues are earlier and later. (1) Can the clinician have the prognosis conversation at all? If AIS has already failed, the SR1 may never be offered. (2) Starting the claim: PIP line 0800 121 4433; Relay UK; BSL VRS via InterpretersLive (connect.interpreterslive.co.uk/vrs?ilc=DWP), Monday–Friday 09:00–17:00. DWP also has a departmental BSL VRS product (Dals / InterpretersLive) limited to DWP numbers. (3) If SREL is refused, the claim falls onto normal rules and assessment providers must accept a BSL interpreter request. Capita’s PIP VRS page runs telephone assessments for BSL users through registered-interpreter VRS. (4) After death: Locked Out documents register-office and coroner interpreter refusal — local-authority / coroner territory, not DWP SREL.

GOV.UK, Get benefits if you’re nearing the end of life. <https://www.gov.uk/benefits-end-of-life>

GOV.UK, The Special Rules for end of life: information for healthcare professionals.

<https://www.gov.uk/government/publications/dwp-factual-medical-reports-guidance-for-healthcare-professionals/the-special-rules-how-the-benefit-system-supports-people-nearing-the-end-of-life>

GOV.UK, Claiming PIP if you’re nearing the end of life. <https://www.gov.uk/pip/claiming-end-of-life>

PIP official statistics to April 2026 — 99% of SREL claims awarded.

<https://www.gov.uk/government/statistics/personal-independence-payment-statistics-to-april-2026/personal-independence-payment-official-statistics-to-april-2026>

ABSENT. No published DWP case, FOI series or PHSO finding that names a Deaf or DeafBlind SREL claimant and finds interpreter failure at a DWP assessment. That absence is structural: accepted SREL claims skip the assessment.

1.4 CQC and PHSO findings that name Deaf or DeafBlind patients

Case	Date	Setting	Finding
Alan Graham, 75, QE Hospital / UHB	11 Feb 2026	Hospital last weeks, heart failure	Interpreters 3 times in 11 weeks; grandchildren 16 and 12 used for prognosis and DNACPR-type conversation
Mrs W, 89, GP practice	20 May 2015	Community prognosis	Profoundly deaf woman not told she was dying in time for a palliative plan
UHB / Good Hope, unnamed 55-year-old Deaf autistic man	Oct 2022 CQC penalty	Hospital consent; died 2020	Two feeding-tube procedures with no attempt at BSL interpreter, family or LPA; £8,000

PHSO summary 1095 (September 2015) records an elderly man deaf on one side with no care planning for that need during an end-of-life admission — unilateral hearing loss, not BSL-using Deaf. CQC rated SignHealth Longley Road outstanding (14 January 2026) — specialist Deaf residential mental-health / learning-disability care, not a hospice, not an EOL communication finding. Baroness Grey-Thompson, Lords 20 March 2026, Terminally Ill Adults Bill: Rosie Cooper’s Deaf father’s last days without a reliable interpreter; St Helena cited as a counter-example; clause 5 requires interpreters in preliminary discussions.

BBC, Birmingham NHS trust fined over deaf patient consent failings, 7 October 2022.

<https://www.bbc.co.uk/news/uk-england-birmingham-63175446>

CQC, SignHealth Longley Road outstanding, 14 January 2026. <https://www.cqc.org.uk/press-release/cqc-upgrades-rating-south-london-specialist-deaf-care-home-outstanding>

ABSENT. *No published hospice inspection narrative names a Deaf or DeafBlind patient whose end-of-life care was marked down specifically for missing BSL or tactile interpreting.*

2. DeafBlind-specific: tactile communication in dying care

Honesty first. The UK DeafBlind end-of-life evidence base is thin to the point of absence on the four questions asked. Sense and Deafblind UK publish method guides and social-care casework. They do not publish a hospice or last-days series. PHSO, CQC and coroners have named Deaf BSL deaths. They have not, in published findings, named a DeafBlind person whose death was investigated because tactile access failed.

Methods in scope

NHS.uk (patient information, not a palliative protocol) lists Deafblind Manual (letters signed onto the hand from a modified BSL fingerspelling alphabet); block alphabet (capitals drawn on the palm); hands-on / tactile signing (receiver's hands rest on the signer's); visual-frame signing; plus residual speech, Tadoma, Moon and Braille. RCSLT adds social-haptic communication (haptics — agreed touch messages on back, arm, shoulder, knee) and bodily-tactile / co-active signing. These are not interchangeable with visual BSL. Remote video interpreting advertised by several hospices is often unusable for a tactile user. Locked Out: in-person provision is required for hand-over-hand translation; VRS/VRI is "often completely inaccessible for this group."

The registered workforce is RIDB (NRCPD Registered Interpreter for Deafblind People), not a generic RSLI and not a communicator-guide. Communicator-guides and intervenors are social-care roles. Government baseline at the 23 June 2026 competency-assessment grant: 8 RIDB for an estimated 12,000 Deafblind people with communication needs; grant aims at 68. Project verification 5 September 2026: live NRCPD RIDB count 19 (London 1; SW / EM / YH 0). That supply figure binds every question below.

NHS.uk, Deafblindness — Management. <https://www.nhs.uk/conditions/deafblindness/treatment/>

Sense, Deafblind Manual, 14 February 2022. <https://www.sense.org.uk/information-and-advice/ways-of-communicating/deafblind-manual/>

RCSLT, Deafblindness clinical information. <https://www.rcslt.org/speech-and-language-therapy/clinical-information/deafblindness/>

GOV.UK, Support for the Deafblind community grows... £15k competency grant, 23 June 2026.

<https://www.gov.uk/government/news/support-for-the-deafblind-community-grows-with-new-funding-to-assess-specialist-interpreters-back-by-the-uk-government>

DHSC, Care and Support for Deafblind Children and Adults, 18 December 2014 (Care Act statutory guidance for local authorities). <https://www.gov.uk/government/publications/deafblind-people-guidance-for-local-authorities>

2.1 Hospice / NHS palliative protocol for tactile communication

Finding: none published as a UK hospice or NHS palliative protocol. AIS requires a deafblind interpreter but does not specify tactile method, last-days practice, or what to do when the patient's hands fail. DHSC 2014 deafblind guidance is Care Act assessment guidance for local authorities — identify, record, specialist assessor, one-to-one support — not an instruction to hospices. NICE NG31 (last days of life)

and NG142 (EOL service delivery) mention professional interpreters generically. Trinity and St Helena published practice is Deaf / hearing-loss / VRI, not tactile.

The one body of practice that does exist is not UK hospice policy. Social-haptic communication, developed by Riitta Lahtinen and Russ Palmer (University of Helsinki group, from the 1990s; Palmer has Usher syndrome). A subset called hospital haptics — a small agreed set of touch messages (blood pressure, do not move, end of treatment, are you ok?, yes/no) — has been used with Deafblind patients during cataract surgery, examinations and isolation (Palmer 2015; Lahtinen, Palmer and Tuomaala, DBI Review, January 2016). Later SHC overviews note haptics have been taught to staff working with older people, people with dementia, and those in palliative care. That sentence is the nearest published claim that palliative staff have been trained in a tactile method. It is Nordic research-practice, not an NHS or Hospice UK standard.

Lahtinen, Palmer, Tuomaala, Using Haptics in Health Care Settings, DBI Review, January 2016. Cited in SHC collected papers / Lived Places and Ca' Foscari SHC volume 2025–26.

Locked Out, 14 urgent improvements for deafblind people, 27 November 2025.

<https://www.gov.uk/government/publications/bsl-user-experience-of-health-and-social-care-in-uk/18-areas-for-action-for-service-providers-and-health-and-social-care-professionals-and-14-urgent-improvements-for-deafblind-people-bsl-and-english-v>

ABSENT. *No English hospice has published an RIDB booking for an inpatient death, a hospital-haptics admission protocol, or a last-days tactile competency.*

2.2 Pain, distress and consent when tactile sign is lost

Hands-on signing and Deafblind Manual require usable hands, arms, attention and energy. Last-days physiology works against that channel: weakness, oedema, neuropathy, MND-type motor loss, sedation, delirium, sleep. For a visual BSL user, “once their eyes are closed, the world is shut out” (Newfoundland Deaf study — Deaf, not DeafBlind, cited only for the mechanism). For a tactile user the equivalent is: once the hands can no longer be offered or read, the language is gone unless a pre-agreed residual code exists. No UK hospice paper describes that residual code.

Closest published method is a US nursing advocacy piece (MyAmericanNurse, 8 September 2023): visual analogue scales fail; suggested substitutes are guiding the nurse’s hand to the site, AAC if already used, vocalisations decoded with family, and the Checklist of Nonverbal Pain Indicators. Not validated in dying DeafBlind adults. Hospital haptics include yes/no and “are you ok?” — they could survive after full signing collapses if taught to patient and night staff before decline. No UK service reports doing that teaching. Court of Protection Oldham MBC v KZ [2024] EWCOP 72 is Deaf BSL capacity, not DeafBlind. A 2021 scoping review of consent strategies for people with deafblindness found no literature specific to this population.

Deafblindness and health advocacy, MyAmericanNurse, 8 September 2023.

<https://www.myamericannurse.com/>

Informed Consent or Assent Strategies... Deafblindness, 2021 scoping review.

<https://www.sciencedirect.com/science/article/pii/S2590109521000197>

ABSENT. *No UK published last-days pain tool, consent protocol or residual-code set for a dying tactile user.*

2.3 Family and interpreter fatigue

Hands-on interpreting is physically continuous contact. Social haptics adds a second stream on the back or arm. A dying trajectory multiplies hours. One RIDB cannot cover that. Family members who already know the person’s tactile system become the default bench. Locked Out urgent improvement 2: increase deafblind interpreters and communicator-guides specifically to avoid reliance on family. Improvement 6:

named preferred interpreter — familiarity treated as a communication requirement. Research priority listed: cognitive load and fatigue related to deafblindness — framed as the person’s load, not the interpreter’s. No UK paper measures RIDB or family hours at a deathbed. Norwegian “Balancing on a knife-edge” (older persons with deafblindness, existential care) is adjacent, not a hospice caseload. Workforce arithmetic is the hard evidence: 8–19 RIDB nationally cannot staff night cover in six SWL boroughs.

ABSENT. Do not import Alan Graham / Francesca Bussey family-interpreter harm and relabel it DeafBlind. The mechanism is the same; the physical load and scarcity of a replacement professional are worse. That distinction is not yet in a published fatigue study.

2.4 Documented DeafBlind deaths without communication access

Item	What it is	What it is not
Locked Out, 27 Nov 2025	Policy statement that hospice is not peaceful if deaf and deafblind people cannot communicate; MND case is a Deaf BSL husband	Not a named DeafBlind death
Deafblind UK, Natalie’s story, 2024	Born Deaf, then glaucoma; tactile hands-on BSL; hospital/rehab staff writing on the arm; moved without warning	Rehabilitation, not dying
Locked Out, woman 49, diabetic retinopathy	Became deafblind; had not understood insulin teaching — no BSL interpreter	Health-literacy harm, not an EOL inquiry
PHSO Alan Graham; CQC UHB 2022; Imogen Nunn PFD	Named Deaf BSL deaths / crises	Not DeafBlind
Trinity / St Helena write-ups	Deaf / VRI access	Not tactile

Deafblind UK, Natalie’s story, 2024. <https://deafblind.org.uk/wp-content/uploads/2024/10/Natalies-story-Case-Study-2024.pdf>

ABSENT. No named UK PHSO, CQC, coroner or inquest case that is DeafBlind + dying + tactile failure.

3. International comparative research

The international literature on Deaf sign-language users in palliative care is small, mostly qualitative, and concentrated in the US, Canada and Germany. The literature on DeafBlind people in hospice is thinner: dual-sensory-impairment papers measure mortality and function in general populations, not hospice quality. Language-discordance palliative reviews (spoken-language minorities) are cited only where the mechanism is the same.

3.1 Outcome disparities

Cerilli C et al., “There is No Communication”: Deaf Signers’ Experiences With Advance Care Planning. Journal of Palliative Care, 2023. USA. Eleven Deaf ASL users. Mean ACP knowledge 4.6 / 15 (SD 2.6). <https://journals.sagepub.com/doi/10.1177/08258597231179763> Transfer: only published ACP knowledge-score in a signing community; a BSL version would give SWL a baseline.

Maddalena V, O’Shea F, Murphy M., Palliative and End-of-Life Care in Newfoundland’s Deaf Community. Journal of Palliative Care, 2012. Canada. No ASL sign for “palliative care”; fingerspelled then glossed as comfort

care as a person approaches death. <https://pmc.ncbi.nlm.nih.gov/articles/PMC4894815/> Transfer: Trinity independently found the same lexical hole in BSL in 2023–26.

Carpenter JG et al., EOL care among older Veterans with hearing and vision loss. JAGS 68:817–825, April 2020. USA (VA). Hearing loss: slightly lower pain-management and communication-satisfaction scores. Vision loss: fewer palliative consults and chaplain contacts. Not Deaf-culture / not tactile. <https://onlinelibrary.wiley.com/doi/10.1111/jgs.16298>

Bright T et al., Prevalence and impact of dual sensory impairment: a scoping review. PLOS Global Public Health, 16 May 2023. DSI associated with higher mortality, poorer health, more depression, worse ADL. Almost none of the studies are hospice cohorts. <https://journals.plos.org/globalpublichealth/article?id=10.1371/journal.pgph.0001905>

Silva MD et al., Language barriers and disparities in hospice enrolment. JPSM 61(3):504–512, March 2021. USA. Spoken LEP. Three mechanisms that also appear in Deaf studies: structural interpreter access; inaccurate translation of EOL concepts; interpreter style during prognosis. <https://www.sciencedirect.com/science/article/pii/S0885392420306886>

Hui-Lio A et al., Language discordance and palliative care: scoping review. PLOS ONE, 2 April 2025. 39 studies. Language discordance reduces access to and quality of PC, hospice and ACP. Almost none are sign-language studies. <https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0321075>

ABSENT. No paper reports a hazard ratio for hospice referral, DNACPR conversation quality, or place-of-death for RIDB/tactile users.

3.2 Documented good practice outside the UK

Allen B, Meyers N, Sullivan J, Sullivan M., ASL and End-of-Life Care: Research in the Deaf Community. HEC Forum 14:197–208, September 2002. USA (Minnesota). Project from 1998–99. Trained Deaf hospice volunteers. ASL educational materials. Information must be in ASL, not translated English. <https://link.springer.com/article/10.1023/A:1020508511133> Transfer: Deaf people trained as hospice volunteers plus materials authored in sign.

Ottawa Deaf Palliative Care Team, from 2009. Ottawa Citizen, 25 December 2016. Canada. Deaf volunteers who took palliative-care courses sit with dying Deaf patients. <https://ottawacitizen.com/news/local-news/deaf-and-dying-how-a-volunteer-team-brings-comfort-through-communication> Transfer: a small on-call Deaf palliative volunteer/advocate team attached to a hospice, distinct from the booked RSLI.

ASL CARES, Jacksonville, Illinois. Journal-Courier, 9 February 2022. USA. Trains Deaf, hard-of-hearing and deaf-blind volunteers as hospice advocates. Role split: interpreter interprets; advocate stays. Founding anecdote: a hospice patient told he had cancer who did not know the word. <https://www.myjournalcourier.com/news/article/Members-of-the-deaf-community-rally-to-assist-16841626.php>

France — UASS / UNISS. Circulaire DHOS-E1-2007-163, 20 April 2007; first unit 1996 Pitié-Salpêtrière; 23 sites by 2023. Hospital units staffed by clinicians bilingual in French and LSF. Model stops at the hospital door when the person chooses to die at home. LSF until recently signed “soins palliatifs” as care as an announced approaching death. <https://www.odella.fr/avant/conditions-de-vie-des-personnes-agees/fin-de-vie/la-prise-en-charge-des-patients-sourds-muets-en-fin-de-vie/>

Germany — Deaf Pal, Hochschule Landshut + LAKUMED Landshut-Achdorf. From January 2021. DGP abstract 2024. CBPR on a palliative ward. Finding: a self-determined death is largely denied because there is no secured communication system. Zeitschrift für Palliativmedizin 25(05):e59–e60. <https://doi.org/10.1055/s-0044-1788489> Transfer: park a BSL advisor inside a hospital SPC team for a defined caseload.

Canada — NLAD + Memorial University ASL palliative video series on Canadian Virtual Hospice; Vancouver Coastal Health Deaf-Deaf-Blind well-being programme videos. <https://www.virtualhospice.ca/> and <https://www.vch.ca/en/deaf-hard-hearing-and-deaf-blind-well-being-program-wbp/resource-videos-with-asl>

Australia — Palliative Care Victoria + Murray PHN + Deaf Hub Bendigo. Auslan pack launched 28 November 2024. Let’s Talk about Palliative Care; Let’s Talk about Death; When Someone Dies; Loss of a Loved One; Loss of a

Loved Pet. Minister for Health on the record. <https://www.healthtranslations.vic.gov.au/resources/palliative-care-information-in-auslan>

United States — ADA effective-communication duty, including family involved in care; ACA §1557; NHPCO culturally sensitive communications guidance (c.2025) tells hospices to film videos in ASL. Statute without a booked bench produces the same failure mode as AIS. <https://www.nhpco.org/>

New Zealand: Hospice NZ Standards 2019 mention sensory disability in one paragraph. No NZSL palliative programme equivalent to the Australian pack was found.

3.3 Prognosis, consent and advance directives through interpreters

Cerilli 2023: even with an interpreter, one participant was handed an EOL brochure with no explanation. Allen 2002: ACP forms have to be designed in sign, then back-translated. Maddalena 2012: study consent itself needed an ASL interpreter and a Deaf interpreter; nodding is not understanding; pain vocabulary does not map. Barwise et al., *Critical Care Medicine* 47(10):1380–1387, October 2019 (Mayo, spoken LEP): pre-meeting between clinician and interpreter; interpretation that carries empathy; bidirectional cultural briefing. KFF Health News, 22 August 2016: some US hospitals embed trained interpreters in the PC team; ACP “is more like psychotherapy than a routine medical consult.” Schenker et al., *JPSM* 2025: 162 interpreters, 27.2% ASL; significantly lower self-efficacy on communication-and-decision-making than on symptom-management. Deaf Pal 2024: relatives become the people who secure communication.

Barwise A et al., EOL decision making for ICU patients with LEP. *Crit Care Med*, Oct 2019.

https://journals.lww.com/ccmjournal/fulltext/2019/10000/end_of_life_decision_making_for_icu_patients_with.13.aspx

KFF Health News, Interpreters Key To Easing Patients’ Final Days, 22 August 2016.

<https://kffhealthnews.org/insurance/more-at-peace-interpreters-key-to-easing-patients-final-days/>

3.4 Family burden when relatives interpret a dying person

Kehl KA, Gartner CM., Challenges Facing a Deaf Family Member Concerning a Loved One’s Dying. 2010. USA.

Deaf father; dying son’s eyes kept closing so visual ASL failed; father would not ask the hospital to pay for an interpreter. ADA theoretically covers family; practice did not.

<https://pmc.ncbi.nlm.nih.gov/articles/PMC2836835/>

Franchi L., Communication experiences of Deaf and hard-of-hearing people facing the death of a loved one.

PhD, Simon Fraser University, 16 December 2011. Canada. Documentary Talkable. Exclusion from dying process and funeral rites compounds grief. <https://summit.sfu.ca/item/12023>

UK cases already in Prompts 1–2 sit on the same shelf. Jurisdictions with a stronger disability statute (ADA) still use the family. The only programmes that reduce the load put a Deaf-trained person in the room as well as an interpreter.

What translates into an NHS / English hospice specification

- 1. A five-part BSL palliative pathway series (Canada + Australia already designed the content). Host on Hospice UK / Marie Curie / NHS.
- 2. Interpreter pre-meet + debrief as a contract line for goals-of-care, DNACPR and last-days conversations. Pay a half-day, not a 60-minute slot.
- 3. Deaf hospice volunteer / advocate bench attached to a named hospice (Minnesota, Ottawa, Jacksonville). Jacksonville is the only brief that writes DeafBlind in.
- 4. ACP knowledge screen in BSL, using Cerilli’s 15 items as a starting instrument.
- 5. A ward-embedded project on the Deaf Pal pattern: one hospital SPC team, a BSL advisor, a defined caseload, a published protocol.

Not worth copying uncritically: ADA-style statute without a bench (the UK already has AIS); VRI tablets as the hospice solution (fails tactile users; fails last days when eyes close); DSI mortality papers as if they were hospice quality metrics. No country in this scan runs a documented tactile/RIDB last-days service.

4. Lived experience, harm, and the case for change

This section is what people said it felt like, what investigators recorded, what charities have asked for in this specific corner of care, and what has already been built that is better than nothing. Anonymised testimony is labelled.

4.1 Formal findings

Alan Graham (PHSO, 11 February 2026) and Mrs W (PHSO, 20 May 2015) are the two named end-of-life ombudsman findings. The UHB CQC penalty (October 2022) is the named consent-without-language finding in a patient who later died. Same Trust as Alan Graham.

Imogen Nunn (Immy), Brighton, died 1 January 2023. Senior Coroner Penelope Schofield, Prevention of Future Deaths 24 March 2025 and 7 October 2025 (ref. 2025-0494). This is not a hospice death. Immy was 25, born Deaf, BSL first language, complex PTSD and personality-disorder diagnoses. The inquest found mental-health service failings contributed. The coroner also found systemic, longstanding challenges in mental health for deaf patients and a national shortage of BSL interpreters that makes it hard to communicate distress in crisis. A welfare visit days before she died went ahead without an interpreter because there was not time to book one. The inquest itself was adjourned two months because no interpreter could be found for Deaf witnesses. Addressees include DHSC, NHS England and NRCPD. Her mother, Louise Nunn: the adjournment was “hugely upsetting... Sadly this is an issue she faced on countless occasions.” Include this because it is the only recent English coroner finding that treats missing BSL as a threat-to-life systems problem. It is not evidence about a hospice bedside.

Judiciary, Imogen Nunn Prevention of Future Deaths report, 7 October 2025.

<https://www.judiciary.uk/prevention-of-future-death-reports/imogen-nunn-prevention-of-future-deaths-report-2/>

4.2 First-person and advocacy-published accounts

Francesca Bussey (BBC, 28 December 2021): one month, two hours of interpreting, crossed-ear sign, shouted at, she signed the sentence that her father was dying. Pearl Clinton (Birmingham Live 24 April 2021; ITV 21 November 2021): at 12, in 2003, asked to tell her Deaf mother her father had died; in 2019 at Heartlands she and cousins took turns while her grandmother was dying. She founded The Sign Life and a petition to stop hospitals using children of Deaf adults as interpreters.

“Having to break such news and interpret to a family member, for example, they have a terminal illness... leaves a lasting impact to their mental health and being disregarded as a family member themselves.”

— Pearl Clinton, 2021

“I am grieving and exhausted and still I have to chase my right to an interpreter, again and again. I look back on the terrible experiences and I still have not recovered.”

— Anonymised widow, Locked Out MND case, 27 November 2025

DeafLink End of Life Report (2016/18), n=31, zero replies from people currently receiving EOL care. One BSL user: “I would rather to die than have too many people come and go and can’t communicate with me at all — I would feel I’ve been left with no dignity.” Counter-example in the same file: a Deaf man who died at home after four and a half years, “a very dignified end of life... well planned, guided and he

was kept informed of every single step... familiar interpreters throughout.” Dignity tracked with planning and a known interpreter, not with the diagnosis.

4.3 Advocacy asks that are actually about dying

Most Deaf-sector health campaigning is AIS, interpreters and mental health. End-of-life specific asks are fewer.

- **Locked Out, 27 Nov 2025.** No specialist BSL provision for palliative and end-of-life care; no strategy which allows deaf people to die with dignity with face-to-face interpreters if too unwell for VRS; hospice is not peaceful if deaf and deafblind people cannot communicate. Rec 8: in-person when bedridden, in pain, EOL or maternity. Rec 12: specialist BSL residential, day and in-home support for older Deaf and older Deafblind people. SignHealth welcomed all 12 recommendations.
- **SignHealth + Help the Hospices, 2013.** Same diagnosis as Locked Out, 13 years earlier; a BSL hospice resource presented as the fix did not become a national pathway.
- **BDA.** Partnership with Culver Law for BSL-accessible will-writing (Remember a Charity Week 2024). Chair Robert Adam: BSL “from cradle to grave.” Wills are not hospice care. They are the one end-of-life legal act the Association has productised in BSL.
- **UK DDPO Monitoring Coalition.** Written evidence TIAB226, 12 February 2025: a duty to provide assisted dying is not matched by a duty to provide palliative care, social care or communication support.

ABSENT. *Sense and Deafblind UK have no published manifesto that is specifically about dying. Locked Out is the vehicle through which both names sit on an EOL paragraph.*

4.4 Emotional and psychological toll

What families describe is not only “we didn’t get an interpreter.” It is being forced out of the role of daughter, son, wife, grandchild and into the role of language. Francesca: pizza versus death. Pearl: grief that cannot start because you said the sentence. The MND widow: trauma in the last months, then the register office, then the coroner. Connor was sixteen. Mia was twelve. PHSO paid them because the distress was the finding. Kehl and Gartner 2010: the dying person and the mourner can both lose the language at the same moment. Franchi 2011: exclusion from dying and from funeral rites compounds grief. Cerilli 2023: ACP knowledge 4.6/15; distrust; peers as the real information source. Locked Out: 95% of BSL users report high stress and low control in service interactions generally — the emotional baseline people carry into the last months. DeafLink: 72% of BSL-user respondents wanted to die at home. There is no published UK psychology paper that measures complicated grief in Deaf or DeafBlind families after an inaccessible death. The testimony is the evidence.

4.5 What improvement has looked like

- **Royal Trinity Hospice, 2023–26.** Deaf-insight training; remote-interpreting devices where night staff can find them; BSL Talk & Tours that turned the missing sign for “hospice” into a conversation. Hospice UK Inclusivity Award, December 2025. One hospice, mostly hearing-loss and visual BSL, not tactile. The only English hospice that has published the work and won a sector prize.
- **St Helena Hospice.** BSL Live button; 24/7 SignVideo for staff. Cited in the Lords. Contact access is not a last-night RIDB. It is better than a crossed-ear sign.
- **DeafLink home-death counter-example.** Familiar interpreters throughout; family kept informed at every step. Dignity as continuity and language.
- **Minnesota, from 1998.** Deaf hospice volunteers; materials authored in ASL.
- **Ottawa, from 2009.** Deaf volunteers who had taken palliative-care courses.

- **ASL CARES, Jacksonville.** Deaf, hard-of-hearing and deaf-blind volunteers as advocates, not interpreters.
- **France UASS/UNISS.** LSF-fluent clinicians inside hospitals, 23 sites. Stops at the hospital door.
- **Canada and Australia video packs.** Five-part ASL series; Vancouver Deaf-Deaf-Blind videos; Victoria Auslan pack with a health minister, November 2024. Marie Curie has one BSL grief film. England has not commissioned the pathway series.
- **BDA + Culver Law, 2024.** A will you can make in BSL.

What the testimony adds up to

The harm people describe is not obscure. A dying person is spoken about, not with. A child carries the sentence. A widow is still booking interpreters for the register office. A doctor who is herself Deaf meets patients on a palliative pathway who do not know they are dying. A coroner tells DHSC and NRCPD that missing BSL in crisis is a threat to life. AIS already forbids it. The lived record is what happens when the duty is not staffed at the hour the conversation arrives.

The improvements that exist share a pattern: someone inside a hospice or a Deaf community decided the conversation could not wait for a national strategy, trained the people in the room, and put language where the night staff could reach it. Trinity, St Helena, Ottawa, Minnesota, Jacksonville, Landshut, Bendigo. That is the case for change as practice, not as a slogan.

The DeafBlind last days are still almost unrecorded. Until a hospice books an RIDB for a death and writes down what the hands did as they failed, that part of the record remains an inference from Natalie's ward and from eight-to-nineteen specialists covering a country.

5. Discovery questions for the SWL file

These are not findings. They are the next asks that would turn this brief into local numbers.

- FOI to the six SWL acute and community providers, SWLSTG, and named hospices (Royal Trinity, Princess Alice, St Raphael's, Shooting Star, Greenwich & Bexley if used by SWL patients, plus any ICB-funded hospice-at-home): RIDB / communicator-guide / Deafblind Manual / hands-on BSL bookings in the last 24 months, by setting and by urgency (planned / same-day / out-of-hours). Fulfilment and cancellation.
- Same FOI: BSL (RSLI) bookings on palliative wards, SPC community caseload, and hospice inpatient units, last 24 months, with out-of-hours split.
- Does any SWL or neighbouring hospice have a written last-days protocol that names tactile methods or hospital haptics?
- Has any English hospice ever booked an RIDB for an inpatient death, and for how many hours?
- SREL / SR1: do SWL trusts and hospices record whether communication support was in place when the prognosis conversation that precedes an SR1 took place?
- Commissioning object worth costing: (a) five-part BSL palliative pathway series; (b) half-day interpreter tariff with mandatory pre-meet for goals-of-care; (c) Deaf hospice volunteer bench attached to one named SWL/London hospice; (d) one ward-embedded Deaf Pal-style project.

6. Source register — principal URLs

Full citations sit in the sections above. This list is the working short-set.

<https://www.england.nhs.uk/long-read/accessible-information-standard-requirements-dapb1605/>

<https://www.cqc.org.uk/guidance-providers/meeting-accessible-information-standard>

<https://www.ombudsman.org.uk/news-and-blog/news/teenager-had-tell-his-deaf-mother-her-father-might-die-after-hospital-failed>
<https://www.ombudsman.org.uk/publications/dying-without-dignity/poor-communication>
<https://www.gov.uk/government/publications/bsl-user-experience-of-health-and-social-care-in-uk/locked-out-exclusion-of-deaf-and-deafblind-bsl-users-from-health-and-social-care-in-the-uk-full-report-bsl-and-english-versions>
<https://signhealth.org.uk/resources/still-ignored-the-fight-for-accessible-healthcare/>
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<https://www.deafblind.org.uk/sites/default/files/uploads/documents/2018/02/End%20of%20Life%20Report%20-%20Deafblind%20only.pdf>
https://spcare.bmj.com/content/3/suppl_1/a69.1

End of working brief. Compiled 8 September 2026 from the four-prompt Grok research batch. Update when FOI returns arrive. Do not treat cream or modelled figures from other SWL files as findings of this brief.